Irinotecan · every three weeks

Supportive-care review sheet · Individualise with the team; not a prescription.

Before treatment

Confirm exact indication, current symptoms, allergies, blood counts, renal/liver function and medicines.

Premedication and nausea

Antiemetics per NCCP/local moderate-risk protocol. Assess prophylactic atropine suitability; separate delayed-diarrhoea plan. moderate emetogenic risk; provide rescue instructions.

Administration

Irinotecan is given on Day 1; no home fluorouracil pump. A repeating 21-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

Skin, nerves and minerals

Report new or worsening symptoms early.

Bowel symptoms

Provide distinct early cholinergic and delayed-diarrhoea plans. Check atropine suitability, including glaucoma/asthma precautions. Diarrhoea must fully resolve before irinotecan; serious symptoms require urgent assessment.

Access and discharge

No ambulatory fluorouracil pump. Confirm venous access, transport, observation and the next visit.

Medication review

Review interactions and supplements with pharmacy. Check CYP3A modifiers and UGT1A1 risk. Discuss fertility, pregnancy, contraception and breastfeeding for the complete regimen.

Contact and follow-up

Give the guide, passport and contact card. Use teach-back for fever/illness, chest or breathing symptoms, severe bowel symptoms and line problems.

> Missing inputs remain unassessed; independent clinical/pharmacy review pending.

Sources: NCCP 00213 · NCCP antiemetic guidance